From: Jiashou Xu
Email: drxu@lvsurgical.com
Phone: (702) 258-7788
Fax: +1 702-442-8641

To: Avery Home Health
Email:
Phone: (702) 416-0611
Fax: +1 702-899-4479

Patient Name: CHARLOTTE ONEIL
DOB: 10/14/1945
Age: 80
Sex Female
SSN Not on file

Phone: (702) 333-1746
Address 6120 MAVERICK ST
Las Vegas, NV 89130
Mail Address Same as address

Primary Insurance
Company: Medicare NV
Plan:
Group #:
Policy #: 6TQ5VJ4PT84
Subscriber: CHARLOTTE ONEIL

Secondary Insurance
Company: Tricare For Life
Plan:
Group #:
Policy #: 00136130403
Subscriber: CHARLOTTE ONEIL

Reason for referral: Wound care referral for packing changes

Diagnoses:

ICD-10 Code	Description
K57.20	Diverticulitis of large intestine with perforation and abscess without bleeding

Provider: Jiashou Xu
Date: 03/17/26 03:14 PM

Patient: CHARLOTTE ONEIL **DOB:** 10/14/1945 **Sex:** F
Provider: Victor Yoon **Visit:** 03/17/2026 11:45AM **Chart:** ONCH07CF30B4

Chief Complaint: Hospital Post-Op

History of Present Illness:

Ostomy productive, tolerating diet. Midportion of wound has opened up.

Physical Exam:

NAD

Soft abdomen, ostomy productive and pink, small ~3cm open of the midline wound midportion and upon probing extends some inferiorly and to the level of abdominal wall which feels intact. No significant purulence.

Assessment:

s/p robotic converted to open LAR, end colostomy

Type	Code	Description
ICD-10-CM Condition	K57.20	Diverticulitis of large intestine with perforation and abscess without bleeding
ICD-10-CM Condition	R10.9	Unspecified abdominal pain
ICD-10-CM Condition	R11.10	Vomiting, unspecified

Plan:

Penrose removed

Some staples removed, but left the majority

Wound care referral for packing changes

Follow up 2 weeks

Type	Code	Modifiers	Quantity	Description
CPT	99024		1.00 UN	POSTOP FOLLOW-UP VISIT

Medications & Allergies:

Allergy	Reaction
codeine	
aspirin	

E-signed by Victor Yoon **on** 03/17/2026 12:16PM PDT
This page was generated at 03/17/2026 12:16PM PDT

Powered by **DrChrono**

colon was sharply excised, and passed off the table with the specimen. The colostomy was then matured in Brooke fashion using sutures of 3-0 Vicryl. Ostomy appliance was placed.

This concluded the procedure. All counts were correct. I was present for the entire procedure.

Signature Line
Electronically Signed By: Yoon, Victor DO
On: 02.20.2026 19:20 PST

Completed Action List:
* Perform by Yoon DO, Victor on February 20, 2026 13:34 PST
* Modify by Yoon DO, Victor on February 20, 2026 19:20 PST
* Sign by Yoon DO, Victor on February 20, 2026 19:20 PST Requested by Yoon DO, Victor on February 20, 2026 13:34 PST
* VERIFY by Yoon DO, Victor on February 20, 2026 19:20 PST

There was a large subcostal incision in the left upper quadrant, and decision was made to enter the abdomen in the right upper quadrant. Incision was made in the right upper quadrant to accommodate a robotic trocar. Veress needle was inserted and the abdomen was then insufflated to a pressure of 15 mmHg. The patient tolerated insufflation well. Veress needle was removed and a robotic trocar was inserted. Robotic camera was inserted and the abdomen was inspected. There were copious abdominal adhesions, there were colonic and omental adhesions to the midline, as well as the incisional hernia noted, I tried to maneuver around the incisions, I noticed that the sigmoid was very densely adherent to its surroundings, decision was made to perform an open procedure. Robotic camera was removed.

Midline incision was made from the epigastric area to the suprapubic area. Subcutaneous tissues were divided using electrocautery, and the midportion of the abdomen, incisional hernia was encountered, and hernia sac was dissected free from its subcutaneous attachments down to the level of fascia. Hernia sac was sharply incised, the contents were reduced, I was then able to incise the fascia and this was achieved by dissecting thickened omental adhesions as well as colonic adhesions from the midline, and then incising fascia in a serial manner. Once the midline was cleared and the fascia had been opened, I was then able to dissect omental as well as small bowel adhesions to the anterior abdominal wall and bring these down. The omentum was then reflected cephalad, Bookwalter retractor was placed. Small bowel was densely adherent to the pelvis, and small bowel adhesions were sharply lysed. There were 2 areas of inadvertent serosal injury, secondary to very dense adhesions, and these were repaired using sutures of 3-0 silk in Lembert fashion.

Small bowel was then retracted cephalad. The left colon was then mobilized along the line of Toldt, colonic mesentery and retroperitoneum were separated along the avascular plane, the sigmoid was noted to be very inflamed, thickened. I mobilized the sigmoid away from the pelvis by using sharp dissection, as it was very densely adherent to this area. The proximal rectum and distal sigmoid were plastered to the pelvis, anterior pelvis, and I needed to perform finger fracture in order to separate the colon from these areas. Once I was able to mobilize the sigmoid, the proximal rectum was also extremely adherent to its surroundings, and I had to further finger fracture this area away from its pelvic attachments. Once this was completed, I was able to reach the soft portion of the rectum, which was in the midportion. A window was created in the mesentery of the descending colon, and the colon divided using a contour stapler and a green load. The mesentery was then taken down to the proximal rectum using Ligasure, some of the mesorectum was divided in order for us to get down to the mid rectum for stapler. Contour stapler was then used to divide the mid rectum, using a green load. The low anterior resection specimen was then passed off the table. Bleeding from the sigmoid mesentery was oversewn using 2-0 silk in figure-of-eight fashion. Hemostasis was satisfactory. The rectal stump was tagged with a 2-0 Prolene.

The left colon was then mobilized further along the white line of Toldt, separating mesentery from retroperitoneum along the avascular plane, up to the level of the splenic flexure. There is significant scarring in the area secondary to the patient's prior left nephrectomy. The splenic flexure was mobilized from attachments using sharp and blunt dissection. The gastrocolic ligament was opened, and this allowed me to fully mobilize the splenic flexure. Hemostasis was satisfactory.

Circular left lower quadrant incision was then made for ostomy site. A column was seen and subcutaneous tissue was sharply excised and discarded. The fascia was incised in a cruciate manner, the rectus muscle was split and the remainder of the abdominal wall was incised using electrocautery. The colon was then brought through the ostomy site, care was taken to avoid twisting. No tension was noted on the colostomy.

Bilateral myocutaneous flaps were then raised so that there could be at tension-free closure. This was done by incising the subcutaneous tissue away from the anterior fascia in all directions. The abdomen was then copiously irrigated and free fluid was suctioned. Hemostasis was satisfactory. 19 French channel drain was placed in the pelvis and left paracolic gutter and exited out a right lower quadrant stab incision. The drain was secured using nylon.

There was still some hernia sac present and this was sharply excised free down to the level of the fascia and discarded. The abdomen was then closed using 2 sutures of 1 looped PDS in running fashion. About every 3 to 4 cm a widely spaced suture of 1 Vicryl in interrupted fashion was placed in order to serve as internal retention sutures, thereby completing our complex closure, and also completing primary repair of the patient's incisional hernia. 4000 mg of Restrata Minimatrix was placed into the subcutaneous space in order to mitigate infection and promote wound healing. Penrose drain was placed at the base of the wound and exited out the inferior portion of the incision. The skin was closed using skin staples. The Penrose drain was secured to the skin using a stitch of Vicryl.

Skin staples were used to close the right upper quadrant incision. Dry clean dressings were placed.

Next we turned our attention to maturing the ostomy. The colon was very thickened at the ostomy, a couple centimeters of

Result type:	Operative Reports
Result date:	February 20, 2026 13:28 PST
Result status:	Auth (Verified)
Result title:	Post Operative Note
Performed by:	Yoon DO, Victor on February 20, 2026 13:34 PST
Verified by:	Yoon DO, Victor on February 20, 2026 19:20 PST
Encounter info:	CHH8101158650, CHH Center, Inpatient, 2/16/2026 - 3/6/2026

* Final Report *

Indication for Surgery
80F with prior history of left nephrectomy and cholecystectomy, presented after recent admission with recurrent diverticulitis. She had intractable pain and failure of medical management and she was taken for OR.

Preoperative Diagnosis
Diverticulitis

Postoperative Diagnosis
Same

Operation
ROBOTIC ASSITED DIAGNOSTIC LAPAROSCOPY, CONVERTED TO OPEN LOW ANTERIOR RESECTION, EXTENSIVE LYSIS OF ADHESIONS 40MINUTES, REPAIR OF SMALL BOWEL SEROSAL INJURY x 2, MOBILIZATION OF SPLENIC FLEXURE, END COLOSTOMY, BILATERAL MYOCUTANEOUS FLAPS, PRIMARY REPAIR OF INCISIONAL HERNIA 5CM DEFECT, COMPLEX CLOSURE WITH INTERNAL RETENTION SUTURES, RESTRATA 4000MG TO SUBCUTANEOUS SPACE

Surgeon(s)
Yoon DO, Victor (Surgeon)

Assistant
Lythgoe, Stacey (NP First Assist)
Presha, Henry (Certified Tech First Assist)

Anesthesia Type and Anesthesiologist
General
Boyle CRNA, Joshua W (Provider)

Blood Loss
Estimate Blood Loss: 100 mL (02/20/26 12:55:00)

Urine Output
500.0 mL

Findings
Copious intra-abdominal adhesions, extremely dense adhesions of sigmoid and proximal rectum to the pelvis, inflamed and thickened sigmoid and proximal rectum

Specimen(s)
Low anterior resection

Complications
None

Technique
The patient was brought to the operating room and laid supine on operating table. General anesthesia was administered. Foley catheter was placed and arms were tucked. Zosyn IV was administered for surgical site prophylaxis. SCDs were utilized for DVT prophylaxis. The patient was prepped and draped in the usual sterile fashion. A timeout was performed verifying correct patient, procedure, site, position, equipment prior to beginning.

FINAL SURGICAL PATHOLOGY REPORT for ONEIL, CHARLOTTE A (SAC26-4648)

LMC PATHOLOGY SERVICES
A Sonic Healthcare Anatomic Pathology Practice

SURGICAL PATHOLOGY REPORT
7455 W. Washington Avenue, Suite 301
Las Vegas, NV 89128
P 702.732.3441
F 702.732.2310

WWW.LMCLABS.COM

CLIA #29D0976712

Accession: SAC26-4648 Collected: 02/20/2026 Time: 11:22am Client: CHHMC - MEDICAL RECORDS
Patient: ONEIL, CHARLOTTE A Received: 02/20/2026 Time: 12:00pm Physician: VICTOR YOON, D.O.
Address: 6120 MAVERICK ST Reported: 02/25/2026 Time: 04:52pm 6900 N DURANGO DR
City: LAS VEGAS DOB: 10/14/1945 Age: 80
State: NV **Zip Code:** 89130-1381 Gender: F LAS VEGAS, NV 89149
PT Phone: (702)333-1746 Client Acc#: Phone: (702) 629-1300 Fax:
MRN: CHH7329836 **Visit #:** NVBDR0-19973455117
Other Phys(s): VICTOR YOON, DO SAROJ LOHANI, M.D. TALHA JILANI, M.D.

SURGICAL PATHOLOGY REPORT

SPECIMEN(s):
COLON, SEGMENTAL RESECTION, LOWER ANTERIOR

SPECIMEN CLINICAL INFORMATION:
CLINICAL HISTORY: Diverticulitis
ICD10 CODE(S): Not provided
PROCEDURE/FINDINGS: Not provided

FINAL DIAGNOSIS:

COLON, LOWER ANTERIOR RESECTION:
DIVERTICULITIS WITH MUCOSAL EROSION, SUBMUCOSAL ABSCESS, AND PERFORATION.
THE MARGINS ARE VIABLE.
NO DYSPLASIA OR MALIGNANCY IDENTIFIED.

Michelle McNamara M.D.
Pathologist
(Case signed 02/25/2026)

MICROSCOPIC:

Microscopic examination performed and the results incorporated into the final diagnosis. All controls stained appropriately.

GROSS DESCRIPTION:

The specimen is received in formalin, labeled with the proper patient identification and "low anterior colon section" consists of 19.3 x 3.4 cm in diameter segment of colon with surrounding pericolonic adipose tissue. The serosal surfaces are gray-tan to brown, diffusely congested with adhesions and there are three, adjacent 0.7 to 2.7 cm transmural defects, which is 3.8 cm to the nearest resection margin. The specimen is sectioned revealing a gray-tan, redundantly folded mucosa with diffuse diverticula and a thickened colon wall with two possible areas of perforation, which are inked black. Additionally found within the container is a 2.4 cm in length x 2.2 cm in diameter additional segment of colon which has a gray-pink and glistening mucosa. The specimen is representatively sampled as follows:
A1-resection margins
A2-transmural defects
A3-A4-diverticula
A5-A6-possible perforation
A7-additional segment of colon (SW)

Technical Component was performed at LMC PATHOLOGY SERVICES 7455W. WASHINGTON AVE, STE 301, LAS VEGAS, NV 89128 CLIA #29D0976712, CAP #7117101 JONATHAN H HUGHES M.D. Ph.D MEDICAL DIRECTOR, Interpretation was performed at LMC 6900 NORTH DURANGO DR, LAS VEGAS, NV 89149 CLIA #29D1072346, CAP #7201251, NOEL YUMIACO, M.D., LABORATORY DIRECTOR,

SURGICAL PATHOLOGY REPORT

Referral Summary

Patient	CHARLOTTE ONEIL		
Patient Previous Name			
Date of birth	October 14, 1945	**Sex**	Female
Race	Unknown	**Ethnicity**	Unknown
Contact info	Primary Home: 6120 MAVERICK ST Las Vegas, NV 89130, US Tel (Cell): +1(702)-333-1746	**Previous Address**	address not available
Preferred Language	English		
Patient IDs	131018932 2.16.840.1.113883.3.7621		
Document Created	March 17, 2026, 18:14:14, EST		
Care provision	from November 17, 2025, 12:44:50, EST to March 17, 2026, 18:14:14, EST		
Performer	Victor Yoon		
Performer	Gerome Villapando		
Author	Jiashou Xu, Sunset Clinic		
Contact info	Work Place: 8930 W Sunset Rd, Suite 300 Las Vegas, NV 89148-5013, US Tel (Work Place): +1(702)-258-7788		
Information recipient	Jiashou Xu		
Document maintained by	LV Surgery Associates PLLC		
Contact info	Work Place: 6850 Durango Drive Suite 218 Las Vegas, NV 89149, US Tel (Work Place): +1(702)-258-7788		

Table of Contents

- Allergies, Adverse Reactions, Alerts
- Encounters
- Immunizations
- Instructions
- Medical Equipment
- Medications Administered
- Medications
- Insurance Providers
- Assessments
- Plan of Care
- Health Concerns Section
- Goals Section
- Problems
- Procedures
- Lab Test Results
- Reason for Referral
- Social History
- Vital Signs

Show all sections

Allergies, Adverse Reactions, Alerts

Type	Substance	Reaction	Status	Date	Severity	Notes
Drug Allergy (416098002)	aspirin (1191)		Active	03/16/2026		

Type	Substance	Reaction	Status	Date	Severity	Notes
Drug Allergy (416098002)	codeine (2670)		Active	03/16/2026		

Encounters

Oct. 26, 2025, 9:50 a.m. Encounter

Provider: Dr. Jonathan Chino - General Surgeon at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: SURGERY CONSULT

Encounter type: 1ST HOSP IP/OBS MODERATE 55

Oct. 27, 2025, 9:55 a.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: PROGRESS NOTE

Encounter type: SBSQ HOSP IP/OBS HIGH 50

Oct. 28, 2025, 9:55 a.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: PROGRESS NOTE

Encounter type: SBSQ HOSP IP/OBS HIGH 50

Oct. 29, 2025, 9:55 a.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: PROGRESS

Encounter type: SBSQ HOSP IP/OBS HIGH 50

Oct. 30, 2025, 10 a.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: PROGRESS SOAP

Encounter type: SBSQ HOSP IP/OBS HIGH 50

Feb. 3, 2026, 3:10 p.m. Encounter

Provider: Sakib Adnan, MD - General Surgeon at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: SURGERY CONSULT

Encounter type: 1ST HOSP IP/OBS HIGH 75

Feb. 4, 2026, 3:15 p.m. Encounter

Provider: Stacey Lythgoe - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: HOSPITAL PROGRESS

Encounter type: SBSQ HOSP IP/OBS MODERATE 35

Feb. 5, 2026, 3:20 p.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: HOSPITAL PROGRESS

Encounter type: SBSQ HOSP IP/OBS MODERATE 35

Feb. 6, 2026, 3:20 p.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: HOSPITAL PROGRESS

Encounter type: SBSQ HOSP IP/OBS MODERATE 35

Feb. 16, 2026, 3:20 p.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: SURGERY CONSULT

Encounter type: 1ST HOSP IP/OBS HIGH 75

Feb. 17, 2026, 9:25 a.m. Encounter

Provider: Dr. Jun Xu - Vascular Surgeon at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Care Team Member: Custom - Dr. Jun Xu
Custom - Michael Kupfer

Reason for visit: HOSPITAL PROGRESS

Encounter type: SBSQ HOSP IP/OBS MODERATE 35

Feb. 18, 2026, 9:25 a.m. Encounter

Provider: Dr. Jonathan Chino - General Surgeon at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: HOSPITAL PROGRESS

Encounter type: SBSQ HOSP IP/OBS MODERATE 35

Feb. 19, 2026, 9:25 a.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Care Team Member: Custom - Michael Kupfer
Custom - Victor Yoon

Reason for visit: HOSPITAL PROGRESS

Encounter type: SBSQ HOSP IP/OBS MODERATE 35

Feb. 20, 2026, 9 a.m. Encounter

Provider: Victor Yoon - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: Surgery

Feb. 20, 2026, 10 a.m. Encounter

Provider: Stacey Lythgoe - Other at Centennial Hills Hospital Inpatient (6900 North Durango Drive Las Vegas, NV 89149, (702) 258-7788)

Reason for visit: Surgery

March 17, 2026, 11:45 a.m. Encounter

Provider: Victor Yoon - Other at Cathedral Rock Clinic (7200 W Cathedral Rock Dr Suite 130 Las Vegas, NV 89128, +1 702-258-7788)

Care Team Member: Custom - Gerome Villapando
Custom - Victor Yoon

Reason for visit: Hospital Post-Op

Encounter Status: Complete

Immunizations

Vaccine	Date	Notes	Status
No immunizations recorded.			

Instructions

	Directive
Instruction excluded/not available	

Medical Equipment

Implanted	UDI	Assigning Authority
No medical equipment recorded.		

Medications Administered

RxNorm Code	Type	Medication	SIG	Status	Date Started
No administered medications reported					

Medications

RxNorm Code	Route	Frequency	Dose	Medication	SIG	Status	Date Started	Eprescribed
No medications reported								

Insurance Providers

Payer name	Policy type / Coverage type	Policy ID	Covered party ID	Policy Holder
Medicare NV			6TQ5VJ4PT84	Self
Tricare For Life			00136130403	Self

Assessments

Name	Date
No assessments recorded.	

Plan of Care

Plan Type	Description	Planned Date
Patient clinical instruction	No Information	
Patient education	No Information	

Diagnostic Imaging Report

Test Code	Code System	Name	Comments	Date
No Information				

Referrals and Future Appointments

Reason	Provider	Location	Planned Date
Follow-up Appointment	Victor Yoon, tel:+1 702-258-7788	7200 W Cathedral Rock Dr Suite 130 Las Vegas, NV 89128	April 7, 2026

Health Concerns Section

No Health Concerns Information

Goals Section

No Known Goals

Problems

SNOMED-CT Code	ICD-CM Code	ICD-CM Version	Problem	Effective Dates	Problem Status
	K57.20	10	Diverticulitis of large intestine with perforation and abscess without bleeding	11/17/2025	active
	R10.9	10	Unspecified abdominal pain	02/17/2026	active
	R11.10	10	Vomiting, unspecified	02/17/2026	active

Procedures

Procedure	Date	Status
99222 1ST HOSP IP/OBS MODERATE 55	Oct. 26, 2025	Completed
99233 SBSQ HOSP IP/OBS HIGH 50	Oct. 27, 2025	Completed
99233 SBSQ HOSP IP/OBS HIGH 50	Oct. 28, 2025	Completed
99233 SBSQ HOSP IP/OBS HIGH 50	Oct. 29, 2025	Completed
99233 SBSQ HOSP IP/OBS HIGH 50	Oct. 30, 2025	Completed
99223 1ST HOSP IP/OBS HIGH 75	Feb. 3, 2026	Completed
99232 SBSQ HOSP IP/OBS MODERATE 35	Feb. 4, 2026	Completed
99232 SBSQ HOSP IP/OBS MODERATE 35	Feb. 5, 2026	Completed
99232 SBSQ HOSP IP/OBS MODERATE 35	Feb. 6, 2026	Completed
99223 1ST HOSP IP/OBS HIGH 75	Feb. 16, 2026	Completed
99232 SBSQ HOSP IP/OBS MODERATE 35	Feb. 17, 2026	Completed
99232 SBSQ HOSP IP/OBS MODERATE 35	Feb. 18, 2026	Completed
99232 [57] SBSQ HOSP IP/OBS MODERATE 35	Feb. 19, 2026	Completed
49320 [59] DIAG LAPARO SEPARATE PROC	Feb. 20, 2026	Completed
44146 PARTIAL REMOVAL OF COLON	Feb. 20, 2026	Completed
44139 MOBILIZATION OF COLON	Feb. 20, 2026	Completed
15734 [LT] MUSCLE-SKIN GRAFT TRUNK	Feb. 20, 2026	Completed
13160 [59] SEC CLSR SURG WND/DEHSN XTN	Feb. 20, 2026	Completed
49594 [59] RPR AA HRN 1ST 3-10 NCR/STRN	Feb. 20, 2026	Completed
15271 [59] SKIN SUB GRAFT TRNK/ARM/LEG	Feb. 20, 2026	Completed
15734 [RT] MUSCLE-SKIN GRAFT TRUNK	Feb. 20, 2026	Completed
49320 [AS:59] DIAG LAPARO SEPARATE PROC	Feb. 20, 2026	Completed
44146 [AS] PARTIAL REMOVAL OF COLON	Feb. 20, 2026	Completed
44139 [AS] MOBILIZATION OF COLON	Feb. 20, 2026	Completed
15734 [AS:RT] MUSCLE-SKIN GRAFT TRUNK	Feb. 20, 2026	Completed
13160 [AS:59] SEC CLSR SURG WND/DEHSN XTN	Feb. 20, 2026	Completed
49594 [AS:59] RPR AA HRN 1ST 3-10 NCR/STRN	Feb. 20, 2026	Completed
15271 [AS:59] SKIN SUB GRAFT TRNK/ARM/LEG	Feb. 20, 2026	Completed
15734 [AS:LT] MUSCLE-SKIN GRAFT TRUNK	Feb. 20, 2026	Completed
99024 POSTOP FOLLOW-UP VISIT	March 17, 2026	Completed

Lab Test Results

No lab test results

Reason for Referral

Reason	Referred By Contact	Referred To Contact

Reason	Referred By Contact	Referred To Contact
Wound care referral for packing changes	Jiashou Xu 10040 ALTA DRIVE SUITE 210 Las Vegas, NV 89145 +1 702-589-9250	Avery Home Health , (702) 416-0611

Social History

Social History Element	Effective Dates
Birth Sex: Female	

Vital Signs

No vitals recorded.